

[Portion Redacted]

Interviewer 02:42

All right. And again, they just want to get a better sense of your clinical practice right now. When your clients come to you. What is usually the presenting issue or their event that they seek your service? I know there's going to be a broad range. But are there typical presenting issues?

Therapist 02:59

Yeah, I think the main problem between couples is, oh, maybe maybe that's not the answer to your question. They come with problems that of communication, that they don't understand each other, and they end up in fights. And, yeah, so that, and that, yeah, they don't understand they or they have trust issues, but they not always know that they have trust issues. But I think that's the, the main cause, or the main problem in relationships. If if that's the case, that one usually one of the two is not trusting the other. And but he's not aware that he's not trusting the other because he thinks I have all the proof. Here's the proof. But it's in the head of the person who doesn't trust the other often, and it's not, not the other person, that's doing the wrong things, but it's the interpretation of the person with the trust issues. That's what I that's a very important thing to find out if that's going on in the relationship is someone not trusting the other and then of course, you want to have the facts but you don't know. I'm not there. So I don't know the facts. So I have to use the communication or the the way that they treat each other. And if they, yeah, nonverbal and verbal and how they respond, and if there is empathy, but maybe I elaborate too much. So

Interviewer 05:08

no, but can you tell me as well sorry, because I'm not very familiar with PIR therapy. Is that correct? Yeah. So the example with trust issues like these, like, what are the common steps that you would take a couple through throughout the duration of the therapy?

Therapist 05:30

Well, first, they have to see the dynamic between them. And they have to realize that's what I would ask that your partner is not the problem. You do? If there is something you have yourself? Yeah, you can only change yourself. So you have to do something about your response to your spouse. And even if he is yelling at you, or doing something or cheating, it can be the case, but then it's important. How do you react to that? And, and maybe you have to leave him? Or but if you don't leave, but get angry, angry, angry all the time. That's not. So you have to see what your responses to the behavior of your partner. And that's where you can do something, also with your feelings, because we all want to feel good. So that's eventually that's what it where it will lead to that. But I don't know if that's the answer to your question. Because the steps you asked,

Interviewer 06:52

which, that's, that's perfect for me, I can visualize it already. It feels like it's very, as you were saying earlier, it seems to combine modern theories of therapy. So for me, it does tend to be like the emotion focused therapy as well. About your feelings of other dynamics. And they're really Yeah.

Therapist 07:11

Much like that. Yeah,

Interviewer 07:14

yes. All right. Perfect. And do you do a formal assessment of the couple when they come in?

Therapist 07:23

Yeah. I have a questionnaire. Yeah. Questions? Is that an assessment? Yeah. I don't know.

Interviewer 07:47

Is this questionnaire is this a formal questionnaire as well? Or did you make this within your own practice? Only? The questionnaire??

Therapist 07:58

Oh, yeah. Now it's an a PRI questionnaire,

Interviewer 08:08

the scale have a name?

Therapist 08:12

No, no,

Interviewer 08:15

though, would you? Would you mind letting me know like the types of questions that are being asked here?

Therapist 08:22

the type? Oh, ah, so I mentioned a couple of questions. Yeah. Well, I Oh, well, it's background information and education and age and all that kind of questions. Ah, what did I say? Well, I also use and what recently happened that that made you come to therapy? And yeah, what you want to solve? And if you use drugs or medication, sleeping behavior or other physical, the physical condition, and then contra indications. It was also released. Yeah. Did you have psychiatric issues or suicide, things like that? Addiction. Conflicts with people also. aggressive behavior. Yeah.

Interviewer 09:41

How long does this questionnaire take to fill out usually with you know?

Therapist 09:52

Think 15 minutes.

Interviewer 09:55

All right. And are there other relationships scale scales that you use as the PIR scale that you're currently using,

Therapist 10:04

no, I don't have a scale.

Interviewer 10:07

All right, then this PIR questionnaire that you have, how informative do you find this to be?

Therapist 10:24

I don't know. It's necessary you need to know, background information and how old someone is. Or what do you want to solve? Yeah. For every therapy, these questions, I guess so maybe, yeah.

Interviewer 10:45

All right. If it's alright, with, you can ask, How much do you pay for these skills? And how much it costs
You

Therapist 10:53

know, I don't pay for them. don't cost anything.

Interviewer 10:57

Okay. So if aside from these PIR, how do you if it's not a formal assessment? How do you usually do the sort of informal assessment of getting to the root of the problems of the couple?

Therapist 11:16

That's, um, with questions? It's i the behavior of the one person I take to ask the other one, what triggers? What's the trigger? And what is the the perception, the little detail that's triggering? I want to find that that's important. Because there is the key to the what? What's bothering the other. So and the meaning, the meaning of that, because it has a meaning. What if someone never cleans this? Well? What's this coffee mark in the kitchen, every day is a very small thing. But that has a meaning for the other one. And we have to find that. Because that's saying, probably something about the person who's bothering who's bothered, but also but the other one, well, why is this for you? And for both an issue, why can you get out of work? Can you find a solution? It's also there must be solution to this problem. Why is it so hard for you both? Well, so that's how I work but that's maybe I understand you want to find? Yeah, no? Well, this

Interviewer 12:54

Doesn't make sense. Yes. All right. So across the years, as well, different therapists and different researchers have characterized the construct of relationship quality quite differently from each other. I wanted to get your view about it for you, personally, and professionally. What is relationship quality?

Therapist 13:19

Quality, the quality of the relationship you mean when is it good relationship? To me? What I think that's yeah, that's your question.

Interviewer 13:32

That's right. Yes

Therapist 13:37

Good relationship is when you understand that when your partner is being hurt, and you can have empathy for that for that he's not perfect.

Interviewer 14:00

Interesting. So for you what differentiates couples with high relationship quality and those with low relationship quality?

Therapist 14:11

Sorry,?

Interviewer 14:14

for you what differentiates couples with high relationship quality? If there is no relationship quality?

Therapist 14:21

I didn't. I didn't hear that. One word

Interviewer 14:25

differentiates what makes

Therapist 14:27

differentiates the couples with hi oh, yeah, yes. Um, they don't. Yeah, they don't take the perspective of the other person. They can't leave their own their own perspective. They can't have the empathy for the other because they it's not their fault, but they don't Don't they only see their own perspective.

Interviewer 15:05

Interesting. Okay. And in your professional experience, what are the most important things in a high quality relationships?

Therapist 15:17

Well, I think I've already checked it in you

Interviewer 15:20

I know it is for you.

Therapist 15:22

But in high quality, well, yeah. Understanding the other person, so and the understanding that you have your own needs and points of view and feelings. And but yeah, the person is someone else and he has his own. Yeah, that can be different from yours. In the end, he try to understand that. And that's, that's, that can be very. Yeah. Nice to do a connection.

Interviewer 16:00

And that's how you conceptualize relationships differ from how your clients conceptualize relationships.

Therapist 16:12

So how I

Interviewer 16:16

refer you a high quality relationship is when you understand when your partner is being heard. We

Therapist 16:23

have a different difference from how my clients see it. Exactly. Yeah. Okay. Oh, yeah, my clients they want, they think they have to, it has to be the same. Their partner has to be like them, or they have to change. You have to change your partner and your partner has to do everything for you, or, and then you have a good relationship. So yeah, it's more a bit selfish. Yeah. We all are selfish, but it's more Yeah. So they Yeah.

Interviewer 17:06

And how does this usually work? In the therapy when you have they conceptualize their relationships like that? And of course, you conceptualize relationships with empathy? How then does this play out into how you how they undergo therapy? Hmm.

Therapist 17:25

Yeah, I explain. It's not either. It's either you or your spouse, it's, you can have both different personalities, different feelings, different needs, that can both exist. It's, we have to be the same or he has to do what I want and otherwise. So yeah, I explained that. And I try to have let them experience it by feeling the empathy for the other and then that it feels okay afterwards, that it is both Okay. Huh? Yeah. So they have to feel that to divide the room or the Yeah. Room for the other for both.

Interviewer 18:28

That's very interesting for me, because it's easy, I think, to explain it just verbally that this is going to happen. But to let them feel that empathy for the other half would be more difficult. How do you usually get them to feel that how do you get them to a place where they feel really feel that empathy for the other?

Therapist 18:48

Yeah. By asking about the triggers, and the meaning behind the trigger, what that means for them, and then if they get emotional about it. And often, it's about something that happened before in their life in their life. And if the other person can understand why his or her spouse is reacting this way, like as a child, his mother didn't pay much attention to him and that's why he has all those needs or you want for me to see him or give him attention. Then the comes Yeah, empathy then then the other one can see it can feel the the sorrow or the pain From the other and Yeah. Then it. It's like, okay, or, Oh, no, I understand that it's not about me. And it's not about me, it's something from your past, I can understand that. That's what I tried to do. And it's not always easy. But once that happens, yeah.. And they understand the partner better, because they understand why he is the way he is.

Interviewer 20:40

All right. And their relationship is a complex system, as we all know, it involves love, sex, identity, relationship maintenance, conflict, satisfaction, etc. During therapy, how do you decide which one to focus on? Or which one in this complex system to target?

Therapist 21:03

It's what clients bring in. Yeah, so. Um, yeah, that's why I don't have that questionnaire to ask all these things. Um, yeah. So if they say, we fight too much, then I focus on that fighting and what's what's happening there. And of course, sometimes I ask other questions. Do you ever have sex? Or do you? Yeah. What are the other things that I want to know that because you want to have a big I wanted to have a picture of their lives. But I, that's yeah, intuitively, intuitively. But I, I want to have a picture of their life. And then I asked the questions, then I need to have the picture and see the fighting in that picture. So when does it happen? Is it only in the weekends? Or is it when you come home from work? Or what? What do you do? And then I asked other questions, if necessary, and if not necessary, I don't ask those questions.

Interviewer 22:29

Interesting. All right. And then, at the end of it, how do you determine when to end a couple therapy?

Therapist 22:37

Mm hmm. When they can manage on their own? When they know, okay, this is our dynamic. Yeah, we sometimes still have that fight. But if that happens, we know what to do. And maybe you have to take a couple of minutes or, or longer. And I know that I shouldn't shout anymore, and then take a breath. And then, ya know, what my own what is happening to me, that's what I need to know what's happening to me. And what can I do instead of blaming my partner?

Interviewer 23:27

And so from that start, when they first see you to eventually ending the therapy? How long does that usually take? I know, there's going to be a broad range. But typically, how long does that take?

Therapist 23:44

from start to end? Yeah. Yeah. Yeah, it's a great deal. Um, but yeah, if they really want to solve their problem, yeah, I guess they take my take a year.

Interviewer 24:11

And in that one year, is that usually once a week?

Therapist 24:21

once every two weeks.

Interviewer 24:22

Once every two weeks? All right. All right. So actually, we are, I believe I mentioned this in the email. We are currently developing a relationship quality scale that hopefully better represents a relationship

as a complex system. And that would potentially hopefully help therapists as well in treatment planning and evaluation. How interested would you be in a scale like this?

Therapist 24:51

I'm not so very why oh yeah, well, maybe.

Interviewer 25:13

it's okay. I am not judging at all.

Therapist 25:16

No. No, yeah, I understand. You're a [profession]? Yeah. Oh I think I can't see what need what? I don't know or, and that the skill can answer to me. Because I just want to ask it or yeah yeah, maybe it's questions or it could be it could be helpful to have all already the answers to the questions that I have to ask. I have to ask a lot of questions. And then on beforehand, you have to the scale with a questionnaire. So you know, have that that could be helping. Helpful. If questions about quarreling and about Yeah, all those things that I have to ask. It could be useful. Yeah. To have at forehand. So, yeah.

Interviewer 26:55

What questions aside from quarreling? You said that you had to ask a lot of questions. And it would be useful if there were answers already to these questions, like quarreling. What else do you need to ask about your couples usually?

Therapist 27:11

Yeah. Um, how much time they spend together? What they do when they have a quarrel, if they maybe also something about empathy, do you know, your triggers from your spouse or from yourself? Or? Yeah, what's triggering you? Um, oh, intimacy questions, or if there have been? Other Other Yeah. affairs? How many religious relationships you had before? Meet meeting this person? how long you're you're together? How much children? Those things? Yeah.

[Portion Redacted]

Interviewer 33:55

Now before we end, I want to talk to you about how we will handle the data. I just wanted to know which data sharing option you are most comfortable with, because given the recent open science practices, of course, we want this project to be as transparent as possible. So for you One option is transcript portions can be shared, none of which can be identified to you. Another option is that the whole transcript can be shared, which is removed of sensitive and overly identifying details. So it will be anonymized. Although in this option, there is a chance that someone who knows you well, professionally can piece together the details in the transcript, the country that you're working in the PIR therapy, and they can narrow it down among their colleagues to who they believe it is. But again, no sensitive information will be shared. So which of these data sharing options is more comfortable for you? And if you're not comfortable with both, let me know and we can discuss others.

Therapist 35:12

But with your message shares

Interviewer 35:14

or so for example, if we write down the manuscript at the end of the or like in an online supplementary information, for example, when they ask, in the first option in the transcript portion are shared, when asked, for example, how do you determine the end of therapy? So that you would then people would see their responses to that? Or how do you determine what a high quality relationship, they can see the transcript portions of that, if that were, if you were to choose your first data sharing option? Of course, if you open to sharing the whole transcript, and then people would be able to see more of the context?

Therapist 35:58

Which which people with whom are you sharing it?

Interviewer 36:04

If it were, you know, if it were open, then that would be open to the public? If anybody who would be interested?

Therapist 36:18

Yeah. But I don't understand. Okay, anybody first

Interviewer 36:35

The first one, all right. I will also give you your copy of the informed consent with these types of options as well and let me know, which will give you further details about the data sharing options. And maybe then you will appreciate the nuance of whichever of the options and then you can change your your opinion then, if ever, or, you know, still have the same opinion. But right now, let's go with the first data sharing option, whichever transcript portions can be shared. And after I give you the informed consent, you can let me know again, which among everything is more comfortable with you.

Therapist 37:25

Okay,

Interviewer 37:26

All right, perfect. So I think that would be it. If you have questions about anything else about the data sharing, etc, we can always correspond through emails, and I would like to ask as well, if you have resources about PIR because that seems very interesting for me as a school of therapy, and I would love to learn more. Would you be okay with sending me some resources to learn more about that over email?

Therapist 37:54

Yeah, that's okay. Yeah.

Interviewer 37:57

Thank you so much, [Name]. And that would end our session for the day.

Therapist 38:02

Yeah. Thank you. Awesome. Oh. It was so good to do.

Interviewer 38:13

All right. Thank you. Thank you. Thank you and enjoy the rest of your day.

Therapist 38:18

Yes, thank you. You too.